facilitate human trials. Five of these sites were equipped to also carry out seroepidemiology studies which would provide critical information to estimate the spread of the virus in certain communities so that specific, localized policies could be planned. In addition, 35 clinical trial sites in hospitals were also set up to ensure that no time was lost in recruitment and carrying out of trials.

By this time, SII and Bharat Biotech were in advanced stages of Phase III clinical trials and their data was looking solid. NEGVAC engaged extensively with them in real time, along with others such as Zydus Cadila to chart out production capabilities and set achievable vaccination goals. NEGVAC's vaccination strategy was based on two components: the information coming in simultaneously from the manufacturers that vaccine availability would be limited in the first six months due to restrictions in their capacities. Even if capacities were ramped up, which NEGVAC and the GoI did finance, output would not increase overnight. It would take three to four months at the bare minimum to set up additional capacities. At this point, the input from NEGVAC to the PMO, after consultations with SII and BBIL was that by the end of August 2021, around 60 crore doses may be available: about 47 crore from SII and 13 crore from BBIL. The second component in the strategy was to plan who to inoculate on priority such that 30 crore high-risk individuals who had the maximum chance of mortality could get some level of protection before the second wave hit the country. Healthcare workers followed by frontline workers and senior citizens would have to be high up on the priority list once the vaccine drive roll-out began.

NEGVAC was also in close conversation with Pfizer, Moderna, Johnson & Johnson as well as Sputnik. However, it became clear early on that unless we adopted a technology transfer model and began to manufacture large doses of the vaccines within the country, the amount these companies could spare was extremely insufficient in comparison to our demand. Pfizer was willing to give us about 50 million doses and Moderna only about 7.5 million.[2] To top that, Pfizer and Moderna didn't want to conduct bridge trials in India, were quoting a very high price per dose ($20–25) and expected full legal indemnity.[3] In addition to that, the cold-chain logistics for safe transportation of mRNA vaccines was going to be a huge challenge for us. Despite these negatives, NEGVAC was in pursuit to procure the technology and transfer it to an Indian company which would manufacture the vaccines locally. Pfizer denied the request at the time, noting that